



Clinical Practice Guidelines

Inpatient Mobility Aids

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation Wide	All Staff

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Introduction

This guideline outlines the procedures and responsibilities of individuals and the health service organisation in relation to safe patient mobilisation and preventing hospital deconditioning in inpatient settings. It ensures the delivery of a uniform, integrated, multidisciplinary approach with management that is consistent with best practice.

Prolonged immobility is associated with decrease in muscle mass and significant functional decline due to a complex process of physiological changes that can affect multiple systems.¹ Early mobilisation has been demonstrated to be an effective therapeutic intervention for improving patient outcomes, reduce use of hospital resources and improve patient length of stay.² A standardised functional mobility assessment ensures patient and staff safety as well promoting early mobilisation and preventing deconditioning.

Sir Charles Gairdner Osborne Park Health Care Group (SCGOPHCG) operates in accordance with the [NMHS Falls Prevention and Management Policy](#), [SCGOPHCG Falls Management Guidelines](#) and the Australian National Safety and Quality Health Service Standards.³

Risk Statement

Non-compliance with this guideline may:

Breach Legislation	☐	Impact on Patient Quality Care	☒
Breach Policy	☒	Impact on Patient Safety	☒
Breach Professional Standards	☒	Misconduct	☐
Breach SCGOPHCG Mission & Values	☒	Staff Safety	☐

Definitions

Deconditioning	The process by which an individual's physical or mental capabilities decline due to a lack of regular exercise, activity, or stimulation. It can occur in various contexts, including physical fitness, cognitive function, and behavioural habits.
Equipment	Refers to patient care equipment that assists with safe manual handling, patient transfers, and mobility. It includes but is not limited to walking aids, wheelchairs, and hoists. ⁴
Fall	A fall is defined as an event which results in a person coming to rest inadvertently on the ground or floor or other lower level.^{4,5,6,7} This definition includes: • All syncopal events.⁷ • Controlled falls (the patient has not been expected to fall and required assistance to ensure their safety to a lower level). • Rolls out of bed (equipment and bed safety required a review. Patients should not be rolling out of a bed to a lower-level multiple times). • Patient is suspected of having put themselves on the floor. (Injuries can still occur and need to be monitored for).⁶
Functional Mobility	The ability of a patient to change and control their body position within their own environment. This involves moving in bed, sitting down to standing up and walking. ⁴

Functional Mobility Chart	An interdisciplinary chart that displays a patient's current level of functional mobility as assessed by a Nurse, Physiotherapist or Occupational Therapist. (Appendix 2). Known as mobility chart.
Intervention	A strategy identified to be implemented to mitigate a recognised risk factor.
iSoBAR	Clinical handover acronym I dentify, S ituation, O bservation, B ackground, A ssessment, and R ecommendation/ R eadback
Mobility Aid	An assistive device used for ambulation such as a walking stick, walking frame, wheeled walker, or crutches.
Multifactorial Falls Risk Assessment (MFRA)	A comprehensive process to identify strategies when the patient is identified at risk of falling.
Mobility Terminology and Abbreviations	Encompasses the terms used to describe the level of assistance required to promote safe transfers, mobilisation, and participation in daily tasks. See Appendix 1 for all abbreviations ^{8,9}

1.0 Functional Mobility Assessment Procedure

1.1 Inpatients

All patients admitted into the hospitals must have a nursing multifactorial falls risk assessment (MFRA) completed and appropriate interventions documented and implemented as per the comprehensive care plan. Re-assessment should occur post fall, when there is a change in medical condition, with transfer to another area or hospital and weekly

1.2 Functional Mobility Assessment including the Functional Mobility Assessment Tool (HiTH patients are excluded)

The health professional should use clinical judgement to determine the timing of assessment. Do not perform the functional mobility assessment if the patient:

- Is not cleared to mobilise by a medical officer.
- Is acutely unwell.
- Has pre-existing or acute cognitive changes and/or cannot follow commands.
- Has behavioural issues.

Before the assessment explain the process and gain consent of the patient.

All patients must be assessed for suitability to complete assessment prior to it taking place. This includes:

- Review patient notes and treatment orders for mobility limitations.
- Check medical condition and observations (stable BP, HR, SpO2, postural BP (if able)).
- Assess cognition (ability to follow commands).
- Consider previous mobility levels (bed/chair transfers, ambulation).
- Ensure the patient is not in pain or pain is adequately controlled.
- Verify hearing and visual aids.
- Check for attachments (IV, drains, catheters, monitoring equipment).
- Ensure appropriate footwear.
- Remove slip or trip hazards
- Consider environment and equipment setup (eg. bathroom rails, chair heights).

1.3 Functional Mobility Assessment Tool :“Have a Go”. ([Appendix 3](#))

The “Have a Go” Mobility Assessment Tool defines four levels of mobility, each with a physical task to assess strength, coordination, balance, tolerance, and the ability to follow directions:

- **Step 1:** the patient moves in bed, changes position and can sit on the edge of the bed.
- **Step 2:** the patient sits safely on the edge of the bed and reaches their arm past their midline to shake the assessor’s hand, before repeating with the other hand if required.
- **Step 3:** the patient sits upright on the side of the bed, lifts their leg off the ground and extends it at the knee, before repeating with the other leg.
- **Step 4:** the patient shuffles forward to the edge of the bed in sitting, leans forward with their trunk and pushes to stand, whilst maintaining standing balance with or without an aid and/or assistance.
- **Step 5:** the patient can lift one foot off the ground then the other with or without an aid and/or assistance. If so, the patient is then safe to walk.
- Once patients can perform the task, they move to the next step. The process of Steps 1 through to 5 takes two - three minutes to complete and is a tool that nurses can use to assess mobility.
- Provide a mobility aid if appropriate e.g., walking stick, zimmer frame, wheelchair.

1.4 Functional Mobility Chart and Handovers

- Complete the functional mobility chart as per assessment findings ([Appendix2](#))
- Regularly review and update functional mobility charts, especially after a fall or change in patient condition.
- Include date of review and assessor.
- Update the nursing care plan and use the i.Clinical Manager (ICM) clinical handover as needed, including:
 - Required equipment/gait aids.
 - Level of assistance needed.
 - Any medical conditions affecting mobility.
 - Patient concerns about falling.
- Ensure handovers for all disciplines is completed as per [SCGOPHCG Clinical Handover - Communicating for Safety](#).

1.5 Intensive Care Unit (ICU)

As per ICU Departmental Guideline ‘Mobilisation of Critically Ill Patients’.

1.6 Specific conditions

Bariatric patients and patients with orthostatic Hypotension should be reviewed by the Physiotherapist and/or Occupational Therapist to identify any movement difficulties and to assist patients to improve functional mobility. Refer to appropriate guidelines.

- [NMHS Bariatric Patient Management](#)
- [SCGOPHCG Orthostatic Hypotension Guidelines](#)

2.0 Roles of Health Professionals

2.1 Nursing

- Refer to the [Functional Mobility Assessment Procedure](#) and [Appendix 3](#).
- Use the Have a Go mobility assessment tool when a patient is cleared to mobilise.
- Discuss any recommendations with the patient and their family/carers after the assessment.

- Reassess and document any changes in the patient's condition, as their condition can fluctuate and affect their mobility.
- For patients transferred from another facility, review Physiotherapy and Occupational Therapy documentation to identify recent functional mobility status.
- Transfer the functional mobility chart and patient equipment when the patient changes rooms or wards.
- Consult with Physiotherapist and/or Occupational Therapist if unsure.
- Complete and implement the multifactorial falls risk assessment and appropriate interventions.

Allied health will review and assess patients as required. Referrals can be made by medical, nursing, or allied health staff as per the referral process outlined below.

2.2 Physiotherapy

- Assess patients for functional mobility, appropriate gait aids, and/or equipment as soon as practicable.
- Complete the functional mobility chart as per assessment findings.
- Conduct post-fall physiotherapy assessments within two working days, following post-fall guidelines.⁶
- Continue physiotherapy assessments as per inpatient guidelines, including:
 - Bed transfers.
 - Sit-to-stand and bed-to/from-chair transfers.
 - Ambulation.
 - Appropriate gait aid.
 - Balance.
 - Gait speed.
- Assess the patient for an individualised exercise program and handover to relevant team members.

2.3 Occupational Therapy

- Assess patients for functional mobility and appropriate equipment, assistive devices, or seating as soon as practicable.
- Complete the functional mobility chart as per assessment findings.
- Conduct post-fall Occupational Therapy assessments within two working days, following post-fall guidelines.⁶
- Occupational Therapy assessments should include:
 - Bed mobility.
 - Transfers.
 - Functional ambulation or transportation of objects.
 - Wheelchair mobility.
 - Equipment provision for transfers.
 - Appropriate assistive devices for functional mobility or ADL tasks.

2.4 Medical Officer

- Authorise and document the patient's weight bearing status.
- Authorise and document if the patient is clear to mobilise post fall.⁶

3.0 Staff Education

All staff working with patients should receive education in mobilising patients and falls management relevant to their role. Information can be found:

- Falls Management Website HealthPoint.
- Falls Self-Directed Learning Package.

- Falls Study Days and conferences.
- Hospital Equipment Service update.
- Induction and orientation. Further information provided on orientation to the area of employment.
- Mandatory Annual Refresher Study Days (see department educator).
- Manual handling education.

4.0 Patient Education

Patients, their families and caregivers should be provided with information about mobility and staying safe while in hospital. The brochure [Staying Safe While in Hospital](#) provides an overview and advice.

5.0 Referrals

Referrals can be made to members of the multidisciplinary team as required to further assess and support the patient with mobility. These team members may include but not limited to Medical Officer, Physiotherapist, Occupational Therapist, Falls Clinical Nurse Consultant, Dietician, Podiatrist, and Pharmacist.

Any patient deemed at a high risk of falls, injury and/or has had two or more falls in hospital should be referred to the [Falls Management Inpatient Service](#) (FIMIS) for reviews by Clinical Nurse Consultant (SCGOPHCG).

Patient requiring Falls education can be referred to the Volunteer Inpatient Falls Education Service via the Falls Management Inpatient Service.

6.0 Mobility Aids

Hospital Mobility aids should be used within the hospital. Mobility aids can be accessed via the ward and the Hospital Equipment Service (HES).

Aids can also be borrowed from the hospital equipment service of both hospitals while an inpatient and home loaned equipment can be organised via an Occupational Therapist or Physiotherapist referral.

A list of the mobility aids available can be found [here](#).

Compliance, monitoring, and evaluation.

The SCGOPHCG Falls Management Subcommittee (FMSC) will monitor compliance with this policy via:

- SAC 1 investigations
- Post Fall Auditing undertaken by the Falls Clinical Lead and FMSC

Collated data will be presented annually to the FMSC for review and action. Outcome and results will be reported to the Comprehensive Care Standard 5 Committee and included in the annual reports.

Related Documents

- [NMHS Falls Prevention and Management Policy](#)
- [NMHS Clinical Incident Management Policy and Procedure](#)
- [NMHS Risk Management of Hazardous Manual Tasks](#)
- [SCGOPHCG Falls Management CPG](#)
- [SCGOPHCG Communicating for Safety Policy](#)
- [SCGH Footwear For Inpatient Safety](#)
- [SCGOPHCG Cognitive Impairment Policy](#)

- [WA DOH Post Fall Multidisciplinary Management Guidelines 2023](#)

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Appendix 1 Mobility Abbreviations^{8,9}

General	
Amb	Ambulate: The ability to walk. This includes assistance from another person or a mobility aid. ⁴
I	Independent: Patient can mobilise safely with no physical assistance, verbal prompting or set up required. A walking aid may be used.
Supervision	Supervision: The patient is steady when mobilising but has an impairment e.g., cognitive, or visual, or an attachment. Requires assistance for verbal cues/prompting and/or to ensure a safe environment
SB A	Standby Assist (at patients' side): Staff must always be positioned directly next to the patient and ready to assist. The patient demonstrates inconsistent performance and/or can be unsteady when mobilising
1A 2A	1 Person Assist and 2 Persons Assist: Hands on assistance from one or two staff is required throughout the transfer, ambulation and/or functional tasks. This may vary from minimal, moderate to maximal assist.
T/F	Transfer: The action of a patient moving from one surface to another. This includes moving from a bed into a chair or moving from one chair to another. ⁴
Bed Mob	Bed mobility: The ability of a patient to move around in bed, including moving from lying to sitting and sitting to lying. ⁴
STS	Sit to Stand: Transition from a seated position to a standing position
SOOB	Sit out of bed: The patient moves from a lying position in bed to sitting on the edge of the bed, or a chair with their feet on the floor.
SOEOB	Sit over edge of bed: The patient moves from a lying position in bed to sitting on the edge of the bed, with their feet on the floor
Weight bearing restrictions	
WBAT	Weight bear as tolerated.: The patient is allowed to put 50% to 100% of their body weight through their affected leg. The actual amount tolerated may vary according to the circumstances
FWB	Full weight bear: The patient is allowed to put their full body weight through the affected leg and ambulate.
PWB	Partial weight bear: The patient is allowed to place a small amount of weight through the affected leg. This is normally documented as a percentage. A mobility aid is required such as a frame or crutches.
TWB	Touch weight bear: The foot or toes of the affected leg may touch the floor (such as to maintain balance) but not support any weight. A mobility aid is required such as a frame or crutches.
NWB	Non weight bear: The affected leg must remain off the ground and not support any bodyweight. The patient may hop on their unaffected leg using a mobility aid such as a frame or crutches.

Equipment	
WZF	Wheeled Zimmer Frame
WF	Zimmer frame
PF	Pulpit frame
4WW	Four wheeled walker
WC	Wheelchair
AC	Axillary crutches
EC	Elbow crutches
G/C	Gutter Crutches
Q/S	Quad Stick
W/S	Walking Stick
S/Sheet	Slide sheet
GF	Gutter Frame
TIS WC	Tilt in Space Wheelchair
MWC	Manual Wheelchair
EWG	Electric Wheelchair

Appendix 2: SCGOPHCG Functional Mobility Chart



Government of Western Australia
North Metropolitan Health Service
Sir Charles Gairdner Osborne Park Health Care Group

Functional mobility chart

Patient name:	Completed by (name & discipline):	Date of assessment:
Treating physiotherapist:		Treating occupational therapist:
Bed mobility	Transfers	
Ambulation	Personal care	

Other (weight-bearing status, medical parameters, specific instructions, falls risk, etc)

Abbreviations

General

T/F Transfer
SB A standby assist
(at patient's side)
1 x A 1 assist
2 x A 2 assist
Min A Minimal assist
Mod A Moderate assist
Max A Maximal assist
STs sit to stand
I Independent

Weight bearing restrictions

NWB Non weight bear
PWB Partial weight bear
FWB Full weight bear
Touch WB Touch weight bear
WBAT Weight bear as tolerated

Equipment

s/sheet slide sheet
F or ZF Frame or Zimmer frame
WF Wheeled frame
WZF Wheeled Zimmer frame
4WW Four-wheeled walker
PF Pulpit frame
GF Gutter frame
AC Axillary crutches
EC Elbow crutches
GC Gutter crutches
Qs Quad stick
Ws Walking stick
WC Wheelchair
Tis WC Tilt in space wheelchair
MWC Manual wheelchair
EWC Electric wheelchair

► This chart is only a guide; seek further assistance if necessary.

► Change the date when updating the chart (physio or nursing staff).

► Keep the patient's nurse bell and walking aid close by the patient.

► Ensure patient wears safe shoes or non-slip socks.

For further information, contact Learning & Development/Occupational Safety & Health/Physiotherapy staff.

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Appendix 3 Have A Go! Mobility Assessment Flow Chart

